NCCP 00254 · VERSION 5

Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or G-CSF order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- A functioning premenopausal ovarian state is not compatible with AI monotherapy. Severe hepatic impairment is not recommended by NCCP; moderate impairment requires caution.
- Offer baseline DXA when starting an adjuvant aromatase inhibitor if not already receiving adjuvant bisphosphonates; assess fracture risk and relevant previous results. Under the 2025 joint guidance, patients with a T-score above −2.0 and no additional risk factors generally have risk and bone density reassessed every one to two years. Patients on bone-protective treatment generally have BMD reassessed about every two years. Fracture, substantial bone loss or a change in risk can require earlier review. The clinician selects the interval and any bone treatment; these are not automatic yearly scans for everyone.
- Assess persistent symptoms and prescribe individually; no automatic prophylactic antiemetic, steroid or G-CSF.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00254 v5 · Monitoring and treatment map

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending